

Policy on Quality Improvement Programme

«Hospital Name»

Document control

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Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» maintains a structured quality improvement programme developed, implemented and maintained by a multidisciplinary committee.

The programme is integrated across the organisation and provides a framework for risk management, ongoing monitoring and improvements based on reviews. It includes quality assurance, process improvement, appropriateness of clinical care, programme review, audits and nursing-care quality.

1. Purpose

The purpose of this policy is to establish the framework for developing, implementing, maintaining, reviewing and updating the quality improvement programme.

This policy does not cover matters outside the quality improvement programme.

2. Scope

This policy applies across «Hospital Name» to the quality improvement programme and its related activities.

It covers management, clinical and support departments, the quality improvement committee, the designated individual, quality improvement champions, doctors, nurses and staff involved in audits and quality improvement activities.

3. Policy standards

3.1 Multidisciplinary quality improvement committee

The quality improvement programme shall be developed, implemented and maintained in a structured manner.

It shall be integrated across the organisation and provide a framework for risk management, ongoing monitoring and performing improvements based on reviews.

The roles and responsibilities of the multidisciplinary committee are defined.

The committee shall have representation from management and various clinical and support departments.

The committee shall receive inputs on significant deliberations from other committees in the organisation.

The committee may be called the core committee, quality improvement committee, etc.

3.2 Quality Improvement Manual

The quality improvement programme shall be documented in the form of a Quality Improvement Manual.

The manual shall incorporate the goals and objectives of the programme, framework for quality improvement activities including data collection, identified indicators, frequency of mock drills, audit schedules, committees and their terms of reference, review policy, and implementation of corrective and preventive action.

Utilisation review preferably could be part of the quality improvement programme.

The quality assurance programme for specific areas like Laboratory, Imaging, Emergency, Blood Centre, Operation Theatre and Intensive Care Unit(s) is also summarised.

3.3 Process efficiency and effectiveness

The quality improvement programme encourages the use of quality tools and novel strategies to improve both clinical and managerial processes.

The impact of managerial process innovations may be at department or organisation-wide level.

Innovations may be targeted to improve patient safety, improve care delivery, reduce costs, or introduce environmentally-friendly measures.

Management promotes these innovations.

3.4 Appropriateness of clinical care

Doctors and nurses participate in utilisation review, including appropriateness of length of stay, procedures, investigations and treatment.

3.5 Designated individual

The designated individual shall undergo a structured training programme in implementation of accreditation standards, with knowledge of statutory requirements, hospital quality improvement principles, evaluation methodologies, and hospital functioning and operations.

The designated individual shall report to Top Management.

Their role and responsibilities shall be defined and documented.

Champions in quality improvement are identified and developed across the organisation and supported to drive improvement.

3.6 Programme review

The quality improvement programme is a dynamic process, with periodic review mechanisms at different levels.

The programme shall be reviewed by the quality improvement committee at regular pre-defined intervals as defined by the organisation, but at least once in three months.

The review shall include findings of audits, organisational performance, and analysis of key indicators.

The minutes of review meetings shall be recorded and maintained.

3.7 Annual review and update

The quality improvement programme is reviewed and updated at least once a year.

The update shall be based on newer literature on quality improvement, audits, feedback mechanisms, and the review carried out by the quality improvement committee.

If the annual review does not identify any opportunities for improvement, that finding itself shall be documented in the committee's minutes.

3.8 Internal audits

Choice and frequency of audits shall be defined for priority areas and areas of concern identified by KPI trends or identified risk.

All areas of the organisation shall be covered by a hospital-wide internal audit at least once in 6 months, per a scheduled plan.

The internal audit shall be done by identified staff or a multidisciplinary team trained in NABH standards, assessing areas independent of their own area of work.

The internal audit shall include all applicable standards and objective elements.

At the end of the audit, there shall be a formal meeting to summarise findings.

Corrective and preventive measures shall be taken and documented.

Implementation of changes is verified and recorded.

3.9 Nursing-care quality

Monitoring shall be done through nursing audits.

Competency evaluation by written questionnaire or witnessed demonstration could also be used for key nursing procedures.

The organisation shall identify key performance indicators reflecting excellence in nursing care, integrated with the quality improvement programme.

4. Non-negotiable rules

1. Do not treat the quality improvement programme as compliant without a multidisciplinary committee developing, implementing and maintaining it.
2. Do not treat the Quality Improvement Manual as complete without its required programme goals and objectives, quality improvement framework, data collection, indicators, mock-drill frequency, audit schedules, committees and terms of reference, review policy, and corrective and preventive action.
3. Do not conduct the quality improvement committee's programme review less frequently than at least once in three months.
4. Do not omit audit findings, organisational performance or analysis of key indicators from the committee review.
5. Do not leave committee review minutes unrecorded or unmaintained.
6. Do not omit the annual review and update of the quality improvement programme.
7. Do not omit documentation in committee minutes when the annual review identifies no opportunities for improvement.
8. Do not conduct the hospital-wide internal audit less frequently than at least once in 6 months.
9. Do not conduct an internal audit of an area using staff assessing their own area of work.
10. Do not complete an internal audit without assessing all applicable standards and objective elements.
11. Do not leave required corrective and preventive measures from an internal audit undocumented.
12. Do not leave implementation of audit-related changes without verification and recording.
13. Do not treat nursing-care quality monitoring as established without nursing audits and identified nursing-care quality indicators integrated with the quality improvement programme.

5. What we do

5.1 Maintain the multidisciplinary committee

The organisation maintains a multidisciplinary committee responsible for developing, implementing and maintaining the quality improvement programme.

The committee has representation from management and various clinical and support departments.

Its roles and responsibilities are defined.

The committee receives inputs on significant deliberations from other committees.

5.2 Maintain the Quality Improvement Manual

The programme is documented in the Quality Improvement Manual.

The manual contains the required goals, objectives, framework, data collection, indicators, mock-drill frequency, audit schedules, committees and terms of reference, review policy, and corrective and preventive action.

Utilisation review preferably could be included.

Area-specific quality assurance programmes for Laboratory, Imaging, Emergency, Blood Centre, Operation Theatre and Intensive Care Unit(s) are summarised.

5.3 Promote process improvement

The programme encourages quality tools and novel strategies for clinical and managerial processes.

Management promotes managerial process innovations.

Innovations may be at department or organisation-wide level and may target patient safety, care delivery, cost reduction or environmentally-friendly measures.

5.4 Review appropriateness of clinical care

Doctors and nurses participate in utilisation review covering appropriateness of length of stay, procedures, investigations and treatment.

5.5 Coordinate the programme

A designated individual coordinates and implements the programme.

The individual undergoes the required structured training, reports to Top Management, and has defined and documented responsibilities.

Quality improvement champions are identified and developed across the organisation and supported to drive improvement.

5.6 Review the programme

The quality improvement committee reviews the programme at regular pre-defined intervals as defined by the organisation, but at least once in three months.

The review includes audit findings, organisational performance and analysis of key indicators.

Minutes are recorded and maintained.

5.7 Review and update annually

The programme is reviewed and updated at least once a year.

The update is based on newer literature on quality improvement, audits, feedback mechanisms and committee review.

When no opportunities for improvement are identified, the finding is documented in committee minutes.

5.8 Conduct internal audits

Audit choice and frequency are defined for priority areas and areas of concern.

All areas are covered by a hospital-wide internal audit at least once in 6 months according to a scheduled plan.

Auditors are identified staff or a trained multidisciplinary team and are independent of their own area of work.

All applicable standards and objective elements are assessed.

A formal findings meeting is held, corrective and preventive measures are documented, and implementation of changes is verified and recorded.

5.9 Monitor and improve nursing care

Nursing-care quality is monitored through nursing audits.

Competency evaluation by written questionnaire or witnessed demonstration could also be used.

Key performance indicators reflecting excellence in nursing care are identified and integrated with the quality improvement programme.

6. Stop-work authority

6.1 Multidisciplinary quality improvement committee trigger

Absence of a multidisciplinary committee developing, implementing and maintaining the quality improvement programme is CORE non-compliance. The absence of this required

committee is a stop-work trigger under this policy and shall be escalated for correction before normal compliance status is treated as restored.

6.2 Nursing-care quality trigger

Absence of an established process to monitor and improve nursing care quality is CORE non-compliance. The absence of this established nursing-care quality process is a stop-work trigger under this policy and shall be escalated for correction before normal compliance status is treated as restored.

7. Governance and responsibility

Role	Responsibility
Management	Provides representation on the multidisciplinary committee and promotes process innovations.
Quality improvement committee	Develops, implements and maintains the quality improvement programme and reviews it at least once in three months.
Designated individual	Coordinates and implements the programme, undergoes structured training, reports to Top Management, and has defined and documented responsibilities.
Quality improvement champions	Are identified and developed across the organisation and supported to drive improvement.
Doctors and nurses	Participate in utilisation review. Nursing-care quality is monitored through nursing audits.
Internal audit staff	Identified staff or trained multidisciplinary teams conduct internal audits independently of their own area of work.

8. Quality monitoring

Monitoring area	What is monitored
Quality improvement committee	Multidisciplinary representation, defined roles, committee inputs and programme review
Quality Improvement Manual	Goals, objectives, framework, data collection, indicators, mock-drill frequency, audit schedules, committees, terms of reference, review policy and corrective/preventive action
Process improvement	Quality tools, novel strategies and managerial process innovations
Clinical appropriateness	Utilisation review by doctors and nurses
Designated individual	Structured training, reporting to Top

	Management and documented responsibilities
Programme review	At least once in three months, including audits, organisational performance and key indicators
Annual update	Review and update at least once a year
Internal audit	Hospital-wide coverage at least once in 6 months and corrective/preventive measures with verification
Nursing care	Nursing audits and nursing-care quality indicators integrated with the programme

9. Training and staff acknowledgement

The designated individual shall undergo a structured training programme in implementation of accreditation standards, statutory requirements, hospital quality improvement principles, evaluation methodologies, and hospital functioning and operations.

Staff acknowledgement:

I have read the Policy on Quality Improvement Programme of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to staff involved in the quality improvement programme.

The quality improvement programme and its relevant processes are communicated across the organisation.

11. Abbreviations

Abbreviation	Meaning
KPI	Key Performance Indicator
NABH	National Accreditation Board for Hospitals and Healthcare Providers
QA	Quality Assurance

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
PSQ.2.a	CORE*	Sections 3.1, 5.1 and 6.1 address the multidisciplinary committee, its responsibilities and the CORE stop-work trigger.
PSQ.2.b	Commitment*	Sections 3.2 and 5.2 address the Quality Improvement Manual and its required contents.
PSQ.2.c	Excellence	Sections 3.3 and 5.3 address quality tools, novel strategies and management promotion of process innovations.
PSQ.2.d	Excellence	Sections 3.4 and 5.4 address doctors' and nurses' participation in utilisation review.
PSQ.2.e	Commitment*	Sections 3.5 and 5.5 address the designated individual, structured training, reporting, documented responsibilities and quality-improvement champions.
PSQ.2.f	Commitment*	Sections 3.6 and 5.6 address committee review at regular pre-defined intervals, but at least once in three months, and required review contents.
PSQ.2.g	Commitment	Sections 3.7 and 5.7 address annual review and update and documentation where no opportunities are identified.
PSQ.2.h	Commitment*	Sections 3.8 and 5.8 address audit planning, hospital-wide internal audit at least once in 6 months, trained independent auditors, findings, corrective/preventive measures and verification.

PSQ.2.i	CORE*	Sections 3.9, 5.9 and 6.2 address nursing audits, nursing-care indicators and the CORE stop-work trigger.
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13. Required Records/Evidence Checklist

Multidisciplinary committee

- Current committee composition showing management and various clinical and support departments.
- Defined committee roles and responsibilities.
- Committee minutes showing inputs on significant deliberations from other committees.
- Documents showing development, implementation and maintenance of the quality improvement programme.

Quality Improvement Manual

- Current Quality Improvement Manual containing goals and objectives, quality improvement framework, data collection, indicators, mock-drill frequency, audit schedules, committees and terms of reference, review policy, and corrective and preventive action.
- Area-specific quality assurance summaries for Laboratory, Imaging, Emergency, Blood Centre, Operation Theatre and Intensive Care Unit(s).
- Utilisation review material where it has been included in the programme.

Process improvement and clinical care

- Quality improvement activities showing quality tools or novel strategies.
- Documents showing managerial process innovations promoted by management.
- Utilisation review material covering appropriateness of length of stay, procedures, investigations and treatment.
- Participation of doctors and nurses in utilisation review.

Designated individual and champions

- Designation of the individual coordinating and implementing the programme.
- Training documentation covering accreditation standards, statutory requirements, hospital quality improvement principles, evaluation methodologies, and hospital functioning and operations.
- Documented role and responsibilities and reporting to Top Management.
- Documentation of quality improvement champions identified and developed across the organisation.

Programme review and annual update

- Quality improvement committee review minutes showing review at least once in three months.

- Review records covering audit findings, organisational performance and analysis of key indicators.
- Annual review and update of the programme.
- Annual review material based on newer literature, audits, feedback mechanisms and committee review.
- Committee minutes documenting the finding when no opportunities for improvement are identified.

Internal audit

- Scheduled audit plan defining audit choice and frequency for priority areas and areas of concern.
- Hospital-wide internal audit records covering all areas at least once in 6 months.
- Auditor details showing identified staff or a multidisciplinary team trained in NABH standards and independent of their own area.
- Completed audit covering all applicable standards and objective elements.
- Formal meeting record summarising audit findings.
- Corrective and preventive measures arising from audits, with documentation.
- Records showing verification of implemented changes.

Nursing-care quality

- Established process for monitoring and improving nursing-care quality.
- Nursing audit records.
- Identified nursing-care quality indicators integrated with the quality improvement programme.
- Competency evaluation records where written questionnaires or witnessed demonstrations have been used.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.2.

Disclaimer

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